GIG CYMRU NHS WALES
Bwrdd Iechyd Prifysgol
Betsi Cadwaladr
University Health Board

Emergency RSI Checklist

Bold points to be read out loud during pre-oxygenation. All team members must participate.

Team

❑ Any additional help required?

Second Anaesthetist

1 - Bleep 063

2 - Bleep 550 / 551

ODP – Bleep 017

(Obs ODP – 510)

❑ Consultant anaesthetist required?

❑ Role allocation:

Team Leader

Intubator 1 and 2

Cricoid Pressure

Intubation Assistant

Drugs

C-spine

Monitor & Time

Patient

❑ Reliable iv access?

❑Cardiovascular status optimal?

Major haemorrhage :give blood

❑GCS and pupil size recorded?

❑ Anticipated complications?

❑ Is position optimal?

Pillow?

Ramping?

Head position?

❑ Is preoxygenation optimal?

❑ High flow nasal oxygen on?

Equipment

❑ Drugs drawn and labelled?

Induction Agents:

state doses: mg, mls

Muscle Relaxant:

state dose: mg, mls

Vasopressor

Maintenance Drugs

❑ Check monitoring:

Capnography (CO_2)

Oxygen Saturations

ECG

BP – 2.5 minute intervals

❑ Intubating equipment:

Intubation Trolley

Videolaryngoscope & blade

Laryngoscopes

2 ET Tubes

Bougie / stylet

Guedel & SAD

Suction to hand

Ambubag or Water's

Difficult Airway Plan

❑ What is the plan for difficult intubation?

Plan A – laryngoscope

Plan B – eg Bag-mask with Guedel

Plan C – eg Supraglottic Airway

Plan D - Cricothyroidotomy

❑ Equipment for above plan readily available?

❑ Is everyone happy to proceed?

Proceed only if Yes

This checklist is not intended to be a comprehensive guide to preparation for induction

V5 MK 2022

Adapted from the 4th National Audit Project of The Royal College of Anaesthetists

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Post RSI & Pre Move Checklist

Bold points to be read out loud post Intubation. All team members must participate.

Airway & Breathing

❑**ETCO2 trace?**

•Consistent >7 breaths

•Peak >1kPa above baseline

•Clinically appropriate

***Target ETCO2* =**

[3.5 to 4.0kPa for head injuries / intracranial bleed]

❑**Oxygen Saturations**

❑**Bilateral breath sounds**

Check tube length.

❑**ETT Length and Secure**

❑ **C-spine secure (trauma)**

Blocks and tape

❑**Sedation running**

If delayed consider rebolus of ketamine or midazolam

Circulation & Disability

❑**Target BP:**

MAP 85mmHg for neuro.

Penetrating trauma: titrate to radial pulse.

❑**At least 2 x iv (or io) access?**

Easy access to flush line.

❑**Pupil size checked & recorded**

❑ **Neuroprotective measures if indicated?**

See separate aide memoire

Equipment

❑ **Drugs to take?**

Extra Muscle Relaxant

Vasopressor

Maintenance Drugs

❑ **Is all monitoring on?**

Capnography (CO_2)

Oxygen Saturations

ECG (3 lead)

BP 2.5 minute interval

❑ **Equipment to take:**

Laryngoscope

BVM

Bougie & ET Tube

Guedel & SAD

Suction

Drugs

Oxygen & Ventilator

Blood / Fluids

Packaging

❑ **Patient packaged on Wolverson mattress (trauma)**

❑ **What is the plan after CT?**

Consider calling EMRTS for transfer

❑**Lines accessible**

❑ **CT ready ?**

❑ **Team members required to prepare equipment post CT?**

Eg: Art line, Urinary catheter, syringe drivers, transfer trolley.

❑ **Happy to Proceed?**

This checklist is not intended to be a comprehensive guide to patient care.

V5 MK 2022

Bwrdd Iechyd Prifysgol Betsi Cadwaladr University Health Board

Paediatric Emergency RSI Checklist

Bold points to be read out loud during pre-oxygenation. All team members must participate.

Team

❑ Any additional help required?

Second Anaesthetist

1 - Bleep 063

2 - Bleep 550 / 551

ODP – Bleep 017

(Obs ODP – 510)

❑ Consultant anaesthetist required?

❑ Role allocation:

Team Leader

Intubator 1

Intubator 2

Intubation Assistant

Drug administer

NGT/PEG aspirator

Monitor & Time

Patient

❑ Reliable iv/io access?

❑Cardiovascular status optimal?

Major haemorrhage :give blood

❑GCS and pupil size recorded?

❑ Known allergies?

❑ Anticipated complications including. cardiovascular?

❑ Is position optimal?

Pillow?

Ramping?

Head position?

C-spine precautions?

❑ Is preoxygenation optimal?

❑ High flow nasal oxygen on?

Equipment

❑ Drugs drawn and labelled?

Induction Agents:

state doses: mg, mls

Muscle Relaxant:

state dose: mg, mls

Vasopressor

Maintenance Drugs ready

❑ Check monitoring:

Capnography (CO_2)

Oxygen Saturations

ECG

BP – 2.5 minute intervals

❑ Intubating equipment:

Intubation Trolley

Videolaryngoscope & blade

Laryngoscopes working

3 ET Tubes: ½ size above & below expected

Bougie / stylet

Guedel & SAD

Suction to hand

Ambubag & Mapleson F

Tube tapes ready

Difficult Airway Plan

❑ What is the plan for difficult intubation?

Plan A – laryngoscope

Plan B – eg Bag-mask with Guedel

Plan C – eg Supraglottic Airway

Plan D - Cricothyroidotomy

❑ Equipment for above plan readily available?

❑ **Is everyone happy to proceed?**

Proceed only if Yes

This checklist is not intended to be a comprehensive guide to preparation for induction

V1 SC 2023

Adapted from the 4th National Audit Project of The Royal College of Anaesthetists

Bwrdd Iechyd Prifysgol
Betsi Cadwaladr
University Health Board

Paediatric Post RSI & Pre Move Checklist

Bold points to be read out loud post Intubation. All team members must participate.

Airway & Breathing

- ❑ **ETT position confirmed by etCO2 trace?**
- ❑ ***ETT length checked & secured?***
- ❑ ***Bilateral breath sounds?***
- ❑ ***Cuff pressure checked?***
- ❑ ***Ventilator settings checked?***
- ❑ ***Target ETCO2 =***

  [3.5 to 4.0kPa for head injuries / intracranial bleed]
- ❑ **Oxygen Saturations**
- ❑ **NGTube in situ**
- ❑ **C-spine secure (trauma)**

  Blocks and tape
- ❑ **Chest X-ray ordered**

Circulation & Disability

- ❑**Target BP:**

  Aim normotension for age.

  MAP >50-60mmHg (CPP 40-50) for neuro.
- ❑**At least 2 x iv (or io) access?**

  Easy access to flush line.
- ❑**Pupil size checked & recorded**
- ❑**Sedation running**

  If delayed consider re-bolus of ketamine or midazolam
- ❑ **Neuroprotective measures if indicated?**

  See separate aide memoire

Equipment

- ❑ **Drugs to take?**
  - Extra Muscle Relaxant
  - Vasopressor
  - Maintenance Drugs
- ❑ **Is all monitoring on?**
  - Capnography (CO_2)
  - Oxygen Saturations
  - ECG (3 lead)
  - BP 2.5 minute interval
  - Temperature
- ❑ **Equipment to take:**
  - Laryngoscope
  - BVM
  - Bougie & ET Tube
  - Guedel & SAD
  - Suction
  - Drugs
  - Oxygen & Ventilator
  - Blood / Fluids

Packaging & Plan

- ❑ **Patient packaged on Wolverson mattress (trauma)**
- ❑ **Patient warming**
- ❑**Lines accessible & snag free**
- ❑**CT ready ?**
- ❑**What is the plan after CT?**

  Consider calling EMRTS or NWTS for transfer
- ❑ **Team members required to prepare equipment post CT?**

  Eg: Art line, Urinary catheter, syringe drivers, transfer trolley.
- ❑ **Happy to Proceed?**

This checklist is not intended to be a comprehensive guide to patient care.

PTO.

Paediatric Post RSI & Pre Move Checklist

Documentation

- **Indication for intubation**
- **Name/grade/specialty of intubator**
- **Size & Length of tube**
- **Number of attempts-state why failed**
- **Laryngoscopy grade & type of Laryngoscope used**
- **CXR reviewed-confirming ETT & NGT/OGT position**
- **Any adjuncts/specific position or equipment used**
- **Any complications/adverse events/issues with patient, equipment**
- **Any other comments.**

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